Medical First Aid (MFA)

196 Practice Questions and Answers for STCW Exams

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Practice Set 1

Q1. Which one of the following should the first provider do when helping a crew with a snake bite?

A) Do not remove jewelry or clothing in the area of the bite
B) If a crew is bitten on the arm, elevate the arm above the level of the heart
C) If the snake is still there, go back and approach from another direction to offer help
D) Suck the venom from the snake bite

Correct Answer: B (If a crew is bitten on the arm, elevate the arm above the level of the heart)

Explanation: Elevating the affected limb above the heart level helps reduce the systemic spread of venom via the lymphatic and circulatory systems. Other options like sucking venom or leaving jewelry on are medically contraindicated as they can accelerate toxin absorption or aggravate swelling.

Q2. What is the best position for a conscious casualty suffering from internal bleeding?

A) Laying down with legs raised
B) Lying flat down
C) Half-sitting position
D) Recovery position

Correct Answer: A (Laying down with legs raised)

Explanation: Laying the casualty down with legs raised (Trendelenburg position variant) promotes venous return to vital organs and improves perfusion to the heart and brain, which is critical in cases of potential internal bleeding or shock.

Q3. The correct ratio of compression to ventilation for two rescuer CPR on a child is:

A) 5:1
B) 5:2
C) 15:1
D) 15:2

Correct Answer: D (15:2)

Explanation: In pediatric resuscitation, standard international guidelines for two-rescuer CPR specify a ratio of 15 compressions to 2 ventilations. This ensures adequate oxygenation and blood circulation for the smaller physiology of a child.

Q4. Shock is best defined as:

A) Inadequate circulation to the body's tissues
B) Inadequate delivery of oxygen to the lungs
C) Inadequate elimination of waste products from the body
D) Inadequate amount of insulin

Correct Answer: A (Inadequate circulation to the body's tissues)

Explanation: Shock is defined as a critical failure of the circulatory system to provide adequate oxygenated blood to the body's tissues. If left untreated, this leads to cellular hypoxia, organ dysfunction, and eventual cardiac arrest.

Q5. When assessing the crew, you should feel for a pulse for:

A) 3 seconds
B) 5 to 10 seconds
C) 15 to 20 seconds
D) 20 to 30 seconds

Correct Answer: B (5 to 10 seconds)

Explanation: Checking for a pulse should be quick but thorough enough to be accurate; 5 to 10 seconds is the recommended window per international first-aid standards. Taking longer delays life-saving interventions, while taking less time increases the risk of a false negative.

Q6. For any victim, the correct compression rate is:

A) At least 120 per minute
B) At least 80 per minute
C) At least 90 per minute
D) 100 to 120 per minute

Correct Answer: D (100 to 120 per minute)

Explanation: International resuscitation guidelines emphasize a high-quality chest compression rate of 100 to 120 compressions per minute for all victims. This range is optimal to maintain adequate cardiac output during external chest compressions.

Q7. One sign that a bandage may be too tight after completing compress bandage is:

A) Blue color of the skin above the bandage
B) Swelling below the bandage and heavy pain
C) Heartbeat increases and a dizzy feeling
D) Swelling above the bandage

Correct Answer: A (Blue color of the skin above the bandage)

Explanation: A blue discoloration (cyanosis) indicates restricted venous return and poor distal circulation, signifying that a bandage is applied too tightly. Immediate adjustment is required to prevent tissue ischemia.

Q8. Sun rays and light reflected from a bright surface (e.g., sea) can cause damage to the skin and eyes. What is this type of burn called?

A) Dry burns
B) Radiation burns
C) Cold burns
D) Electrical burns

Correct Answer: B (Radiation burns)

Explanation: Sunburn and damage from reflected light (such as UV glare from the sea) are classified as radiation burns. These are caused by electromagnetic radiation damaging the skin and eye tissues.

Q9. The treatment of electrical burns is:

A) Remove loose skin and apply ointment. Don't secure with a bandage
B) Apply lotions and ointment to the injured area and secure with a bandage
C) Place sterile dressing over the burn and secure with a bandage
D) Break blisters and secure with a bandage

Correct Answer: C (Place sterile dressing over the burn and secure with a bandage)

Explanation: Electrical burns often involve deep tissue damage that is not immediately visible; applying lotions or breaking blisters increases infection risk. A sterile, non-adherent dressing protects the site until professional medical assessment is possible.

Q10. Asthma attacks can be triggered by:

A) Loud music
B) Too much fresh air activity
C) Not keeping to the diet
D) Nervous tension, allergy, or non-obvious cause

Correct Answer: D (Nervous tension, allergy, or non-obvious cause)

Explanation: Asthma is a complex respiratory condition that can be triggered by a wide array of environmental, emotional, or physiological stressors. Nervous tension, allergens, and infections are common triggers identified in clinical maritime medical guides.

Q11. If a person is in a state of shock, the correct thing to do is:

A) Apply hot-water bottles to keep the patient warm
B) Move the casualty as much as possible
C) Be kind to the casualty and give anything to eat or drink at the first opportunity
D) Treat and reassure the casualty and stay with the person at all times

Correct Answer: D (Treat and reassure the casualty and stay with the person at all times)

Explanation: Treating shock requires ensuring adequate blood flow to vital organs while keeping the patient calm and warm (without overheating). Staying with the casualty provides essential psychological support and constant monitoring for changes in condition.

Q12. When you suspect a casualty has fractured a bone, you should:

A) Raise the affected portion of the body above the level of the casualty's head
B) Massage the affected area to prevent stiffness
C) Rinse the area with cold water
D) Immobilize the affected area

Correct Answer: D (Immobilize the affected area)

Explanation: Immobilization is the priority in fracture management to prevent secondary damage to nerves, blood vessels, and soft tissues. Splinting the joint above and below the fracture site restricts movement and stabilizes the bone.

Q13. If the heart of a casualty has stopped, brain damage is likely to occur after:

A) Beyond 6 minutes
B) Beyond 20 minutes
C) Beyond 1 minute
D) Beyond 10 seconds

Correct Answer: A (Beyond 6 minutes)

Explanation: Once circulation stops, the brain begins to suffer irreversible damage due to oxygen deprivation within approximately 6 minutes. Rapid initiation of BLS is essential to bridge the time until professional care is available.

Q14. The most effective method of artificial respiration is:

A) Silvester method
B) Mouth-to-mouth method
C) Heath Robinson method
D) Mouth-to-nose method

Correct Answer: B (Mouth-to-mouth method)

Explanation: Mouth-to-mouth resuscitation is the most efficient method for delivering the required volume of oxygen to a casualty's lungs. It is the global standard for Basic Life Support when external cardiac support is combined with rescue breathing.

Q15. A resuscitator is:

A) An insulated and heated bag that is used to wrap around a casualty suffering from hypothermia
B) An oxygen tank, with a demand valve and mask
C) An electrical device with two paddles that can be used to restart the heart
D) A plastic tube that fits over the casualty's throat to keep an airway open

Correct Answer: B (An oxygen tank, with a demand valve and mask)

Explanation: In a maritime context, a resuscitator typically refers to an oxygen delivery system that includes a demand valve and a face mask. This equipment provides a controlled flow of medical oxygen to assist a patient in respiratory distress.

Q16. The best way to control severe bleeding is:

A) Raise the bleeding part above the level of the head
B) Direct pressure over the wound
C) Application of a tourniquet
D) Direct pressure on a pressure point

Correct Answer: B (Direct pressure over the wound)

Explanation: Direct pressure over the wound is the primary, most effective method to stop bleeding by facilitating clot formation. It should be the first action taken before considering secondary methods like elevation or pressure points.

Q17. The type of stretcher often found on board is:

A) The canvas pole stretcher
B) The Hart Imco stretcher
C) The SOLAS stretcher
D) The Neil Robertson stretcher

Correct Answer: D (The Neil Robertson stretcher)

Explanation: The Neil Robertson stretcher is a standard piece of maritime rescue equipment specifically designed for the safe vertical or horizontal extraction of injured personnel from confined shipboard spaces.

Q18. The unconscious or recovery position is used to:

A) Ease the pain of broken bones
B) Minimize nose bleeding
C) Prevent the casualty from drowning in his own vomit
D) Correct for any spinal injury

Correct Answer: C (Prevent the casualty from drowning in his own vomit)

Explanation: The recovery position keeps the airway patent and prevents aspiration of vomit or blood should the unconscious patient retch or experience a tongue obstruction. It is a vital safety measure for any non-breathing or unconscious casualty.

Q19. In the absence of a doctor, artificial respiration and heart compression should be continued until:

A) When the casualty turns pale and starts to go cold CPR can be stopped
B) Until the heart starts beating or the rescuer is unable to continue because of fatigue
C) When the casualty shows no response to CPR after 20 minutes, it is useless to continue
D) When the casualty has fixed and dilated pupils for 15 minutes you should stop CPR

Correct Answer: B (Until the heart starts beating or the rescuer is unable to continue because of fatigue)

Explanation: Resuscitation efforts must continue until the casualty regains spontaneous circulation, a medical professional declares them dead, or the rescuer becomes physically exhausted. Premature cessation based on time or visual appearance alone is not permitted.

Q20. The Recovery Position is:

A) The patient is seated in a position with the head kept as low as possible
B) The patient is placed in a 'face-to-the-floor' position with arms and legs arranged in order to stabilize this position
C) The patient is placed flat on a bed
D) The patient is seated in an upright position and with the arms and legs arranged in order to keep this position stable

Correct Answer: B (The patient is placed in a 'face-to-the-floor' position with arms and legs arranged in order to stabi

Explanation: The recovery position, or lateral position, is used for unconscious casualties who are still breathing. This position prevents the tongue from obstructing the airway and allows fluids like vomit or blood to drain, thereby preventing aspiration.

Q21. What do you call the method used for bone-soft part injuries?

A) REHAB-method
B) ABC-method
C) First Aid-method
D) ICE-method

Correct Answer: D (ICE-method)

Explanation: ICE stands for Ice, Compression, and Elevation. It is the gold-standard protocol for managing soft tissue injuries, such as sprains and strains, to reduce inflammation and pain effectively.

Q22. You witness a man getting electric current through his body and is stuck to the dangerous area. It is impossible to switch off the current by any main switch. How do you break the current safely?

A) Stand on dry insulating material and pull the person away with isolating material
B) Just take the casualty in your arms and pull the person away
C) Apply fish oil on your hands and cut the cords by the use of any metallic pliers
D) Call the electrician immediately

Correct Answer: A (Stand on dry insulating material and pull the person away with isolating material)

Explanation: Safety of the rescuer is paramount. Using dry insulating material prevents the electrical current from grounding through the rescuer's body, protecting them from becoming a casualty themselves during the rescue.

Q23. Abdominal Thrust is a technique that involves applying a series of thrusts to the upper abdomen to force air out of a choking casualty's lungs. How do you perform this technique?

A) Lay the casualty on a hard surface, e.g., deck, and press firmly and rapidly on the middle of the lower half of the breastbone
B) Remove the obstruction and restore normal breathing
C) Let the casualty grab a list and hang upside down for a period of a minimum of 5 minutes
D) Stand behind the casualty. Clench your fist with the thumb inwards in the center of the upper abdomen. Grasp your fist with your other hand and pull quickly inwards

Correct Answer: D (Stand behind the casualty. Clench your fist with the thumb inwards in the center of the upper abd

Explanation: The Abdominal Thrust (Heimlich maneuver) is performed by placing a fist between the navel and the sternum and delivering quick, inward and upward thrusts to create an artificial cough and dislodge an airway obstruction.

Q24. When acting as a watcher or lookout at a cargo hold and men below show signs of distress, what must you do?

A) Try to rescue them yourself
B) Raise the alarm immediately
C) Lower additional breathing equipment
D) Don a B.A. set and enter the space

Correct Answer: B (Raise the alarm immediately)

Explanation: A watcher must never enter a space where they suspect a hazard because they may become a casualty themselves. The primary duty is to raise the alarm to mobilize trained emergency response teams equipped for enclosed space rescue.

Q25. A severe blow to or a heavy fall on the upper part of the abdomen (solar plexus) can upset the regularity of breathing. What are the symptoms and signs?

A) The casualty is speaking in a very loud manner
B) The casualty may start sweating profusely and develop a fever
C) The casualty feels very hungry
D) Difficulty in breathing in, and the casualty may be unable to speak

Correct Answer: D (Difficulty in breathing in, and the casualty may be unable to speak)

Explanation: A blow to the solar plexus causes a temporary spasm of the diaphragm, which controls breathing. This results in significant difficulty inhaling and often renders the casualty temporarily unable to speak due to respiratory distress.

Q26. In which way may intake of poisonous material occur?

A) By inhalation
B) Swallowing
C) Skin penetration and skin absorption
D) All mentioned

Correct Answer: D (All mentioned)

Explanation: Poisonous substances can enter the human body via ingestion (swallowing), inhalation of fumes or gases, and direct contact through skin absorption or penetration (such as needle sticks or chemical burns).

Q27. Due to exposure to heat fatigue, heat stroke, and dehydration, what is the maximum recommended effective temperature (ET) for a full workload in enclosed spaces?

A) 30.5 degreesC
B) 27.5 degreesC
C) 29.0 degreesC
D) 35.0 degreesC

Correct Answer: B (27.5 degreesC)

Explanation: Maritime health guidelines state that 27.5 degreesC Effective Temperature (ET) is the safe upper limit for continuous work in enclosed spaces to prevent heat-related illness such as heat exhaustion and heat stroke.

Q28. A casualty suddenly loses consciousness and falls to the ground letting out a strange cry. The patient is red-blue in the face and froth may appear around the mouth. You are witnessing a major epileptic attack. What should you do?

A) Move the casualty into a sit-up position and put something in the person's mouth to protect the tongue
B) Forcibly restrain and try to wake the casualty
C) Loosen tight clothing, ask all unnecessary bystanders to leave, and carefully place something soft under the head. If the casualty is unconscious, place the person in the Recovery position
D) Give the casualty a lot to drink and keep talking to the person at all times

Correct Answer: C (Loosen tight clothing, ask all unnecessary bystanders to leave, and carefully place something so

Explanation: During an epileptic seizure, the goal is to protect the casualty from injury by clearing the area and cushioning the head. Once the seizure ends, placing the person in the recovery position ensures a patent airway.

Q29. Treatment of burns and scalds depends on the severity of the injury. What is the correct thing to do for minor burns and scalds?

A) Break blisters, remove any loose skin or foreign objects from the injured area
B) Place the injured part under slowly running cold water for at least 10 minutes, but preferably until the pain is gone. If no water is available, use any cold, harmless liquid
C) Remove all sticky clothing from the casualty
D) Apply lotions, ointments, or fat to the injury

Correct Answer: B (Place the injured part under slowly running cold water for at least 10 minutes, but preferably until

Explanation: Cooling a burn under clean, cool running water is the most effective way to dissipate heat and limit tissue damage. Applying lotions or fats traps heat and can cause further damage to the wound.

Q30. When acting as a watcher or lookout at a cargo hold and men below show signs of distress, what must you do?

A) Raise the alarm immediately
B) Try to rescue them yourself
C) Lower additional breathing equipment
D) Don a B.A. set and enter the space

Correct Answer: A (Raise the alarm immediately)

Explanation: The watcher's primary responsibility is to maintain communication and raise the alarm immediately if the workers below show signs of distress. Entering the space without the appropriate training and equipment is strictly forbidden.

Practice Set 2

Q1. The acronym SAMPLE stand for signs and symptoms, allergies, medications, past history, ________ and events.

A) Location of pain.
B) Last bowel movement.
C) Last meal.
D) Latest related injury.

Correct Answer: C (Last meal.)

Explanation: SAMPLE is a standard mnemonic for history taking. It stands for Signs/Symptoms, Allergies, Medications, Past medical history, Last meal, and Events leading up to the incident.

Q2. Which one of the following is not a system of the body:

A) Respiratory.
B) Musculoskeletal.
C) Endocrine.
D) Follicular.

Correct Answer: D (Follicular.)

Explanation: The human body consists of major systems like the respiratory, musculoskeletal, and endocrine systems. The term 'follicular' relates to structures like hair follicles but is not classified as a major physiological body system.

Q3. Increased respiratory difficulty accompanied by a weak ineffective cough, wheezing, high-pitched crowing noises and cyanosis are signs of:

A) Mild airway obstruction
B) C.O.P.D.
C) Severe airway obstruction.
D) Complete airway obstruction.

Correct Answer: C (Severe airway obstruction.)

Explanation: These signs (weak cough, wheezing, cyanosis, and high-pitched noises) indicate that the airway is significantly compromised but not yet fully blocked. This requires urgent intervention to prevent total obstruction.

Q4. The most important step in managing shock is to:

A) Keep the patient warm.
B) Give CPR as soon as possible.
C) Give first aid for the illness or injury.
D) Elevate the lower extremities.

Correct Answer: C (Give first aid for the illness or injury.)

Explanation: While keeping a patient warm and positioning are supportive measures, the most critical step in managing shock is treating the underlying cause, such as controlling hemorrhage or managing the primary medical emergency.

Q5. A small percentage of casualties with chronic obstructive pulmonary disease have hypoxic drive. These patients breathe because of a:

A) High oxygen level.
B) Low oxygen level.
C) High carbon dioxide level.
D) Low carbon dioxide level.

Correct Answer: B (Low oxygen level.)

Explanation: Patients with chronic respiratory issues often adapt to high carbon dioxide levels, causing their drive to breathe to shift from CO2 sensitivity to reliance on low levels of oxygen in the blood.

Q6. The emergency responder is one link in the chain of services known as the:

A) Emergency Patient Care (EPC) system.
B) Emergency Medical Services (EMS) system.
C) Professional Emergency Care (PEC) system.
D) Community Medical Care (CMC) system.

Correct Answer: B (Emergency Medical Services (EMS) system.)

Explanation: The chain of survival in emergency response is formally organized under the Emergency Medical Services (EMS) *system, which coordinates care from the initial responder through to hospital arrival.*

Q7. The acronym used to assist the emergency responder assessing the patient's level of responsiveness is:

A) SAMPLE.
B) EMCA.
C) OPQRST.
D) AVPU.

Correct Answer: D (AVPU.)

Explanation: AVPU is the standard scale used in the primary survey to assess neurological function: Alert, Verbal, Pain, and Unresponsive. It provides a quick snapshot of the patient's level of consciousness.

Q8. An industrial worker sustains a severe laceration to his forearm. Direct pressure to the wound fails to control the bleeding. The correct arterial pressure point to control the bleeding is the:

A) Carotid.
B) Radial.
C) Femoral.
D) Brachial.

Correct Answer: D (Brachial.)

Explanation: The brachial artery, located on the inside of the upper arm, is the recognized pressure point to compress when controlling severe bleeding in the forearm or hand that direct pressure alone cannot stop.

Q9. Pressing on a fingernail-bed to observe the return of normal colour is done to check for:

A) Normal blood circulation to that part.
B) The presence of fractured fingers.
C) Pain in the area.
D) A lack of oxygen in the blood.

Correct Answer: A (Normal blood circulation to that part.)

Explanation: Capillary refill (checking the nail-bed) is a simple diagnostic test to evaluate peripheral circulation and perfusion. If color fails to return quickly, it suggests poor blood flow to the extremity.

Q10. Your primary survey of a casualty involved in a serious car collision shows only that he is confused. Later you find his pulse rate at 140 and weak, his skin cold and clammy and his breathing irregular and gasping. These signs, along with the mechanism of injury, indicate:

A) An oncoming faint.
B) Emotional stress.
C) Internal bleeding.
D) Onset of diabetic coma.

Correct Answer: C (Internal bleeding.)

Explanation: Signs of shock (tachycardia, cool/clammy skin, rapid/shallow breathing) in the context of trauma strongly suggest internal hemorrhage, which is a life-threatening condition requiring immediate medical evacuation.

Q11. For which one of the following infectious diseases is a vaccine presently available:

A) Herpes.
B) AIDS.
C) Mononucleosis.
D) Hepatitis B.

Correct Answer: D (Hepatitis B.)

Explanation: Hepatitis B is a viral infection that can be prevented via vaccination. AIDS, Herpes, and Mononucleosis currently do not have effective, widely available vaccines.

Q12. Based on current research, which of the following statements about the AIDS virus is correct?

A) It can be found in blood and semen.
B) It can be transmitted by sharing eating utensils.
C) It can be transmitted by shaking hands.
D) It can be found in perspiration.

Correct Answer: A (It can be found in blood and semen.)

Explanation: HIV/AIDS is transmitted through the exchange of bodily fluids like blood and semen. It is not transmitted via casual contact, such as shaking hands or sharing common items like cutlery.

Q13. In a hazmat situation airway management and immobilzation are carried out in the:

A) hot zone
B) warm zone
C) cold zone
D) neutral zone

Correct Answer: B (warm zone)

Explanation: In a hazmat incident, the 'warm zone' is the designated area where decontamination and initial medical stabilization (including airway management) occur to prevent cross-contamination.

Q14. During the primary assessment of a responsive adult patient, you detect a breathing rate of 28 breaths per minute. You would categorize this as:

A) Above normal.
B) Below normal.
C) Normal.
D) Indeterminate.

Correct Answer: A (Above normal.)

Explanation: The normal respiratory rate for an adult is typically 12-20 breaths per minute. A rate of 28 is clinically considered tachypneic (rapid breathing) and is therefore above the normal range.

Q15. A blood-soaked dressing on the arm indicates that bleeding has not yet been controlled. You should now:

A) Remove the dressing and check the wound.
B) Apply pressure to the femoral artery.
C) Place a clean dressing on top and apply more pressure.
D) Apply a tourniquet.

Correct Answer: C (Place a clean dressing on top and apply more pressure.)

Explanation: Removing a blood-soaked dressing disturbs the clot that is already forming, which can worsen bleeding. It is standard practice to add clean dressings on top and maintain constant, firm pressure.

Q16. The type of shock that is caused by a severe infection is called:

A) Septic.
B) Psychogenic.
C) Cardiogenic.
D) Hemorrhagic.

Correct Answer: A (Septic.)

Explanation: Septic shock occurs when a severe bacterial infection enters the bloodstream, causing a systemic inflammatory response that leads to vasodilation and a dangerous drop in blood pressure.

Q17. Which one of the following is considered a breach of duty:

A) failure to obtain consent.
B) failure to wear your name tag.
C) inappropriate use of lights and siren.
D) insubordination.

Correct Answer: A (failure to obtain consent.)

Explanation: A first responder has a legal and ethical duty to obtain consent before treating a patient. Providing care without consent, except in life-threatening emergencies where the patient is unconscious, is considered a breach of duty.

Q18. Immediately before a seizure, the patient experiences an unpleasant odour. This phase is referred to as:

A) Clonic.
B) Aura.
C) Tonic.
D) Postical.

Correct Answer: B (Aura.)

Explanation: An aura is a sensory warning (such as a specific odor, visual disturbance, or feeling) that often precedes the onset of a tonic-clonic seizure in many patients.

Q19. The emergency responder should assume a head/spinal injury in any unwitnessed situation where the patient is:

A) Alert.
B) Responsive to pain.
C) Not breathing.
D) Unresponsive.

Correct Answer: D (Unresponsive.)

Explanation: In an unwitnessed fall or accident, the mechanism of injury (e.g., a fall from height) and the patient's unresponsive state necessitate that the responder assumes a spinal injury until proven otherwise.

Q20. The secondary assessment of the patient consists of a head to toe examination and a check of the:

A) Pressure points.
B) Procedures for administering CPR.
C) Manual stabilization of the head.
D) Vital signs.

Correct Answer: D (Vital signs.)

Explanation: A secondary assessment is a methodical head-to-toe survey of the casualty, which includes documenting the patient's vital signs (pulse, respiration, blood pressure) to track changes in their condition.

Q21. You are taking blood pressure by palpation. A radial pulse indicates a blood pressure of at least:

A) 110 mmHg
B) 100 mmHg
C) 90 mmHg
D) 80 mmHg

Correct Answer: D (80 mmHg)

Explanation: If a radial pulse is palpable, it is generally accepted that the systolic blood pressure is at least 80 mmHg. If the radial pulse is absent but a carotid pulse is present, it is lower.

Q22. You are called for an asthma attack, treatments may include all except:

A) Metered dose inhaler
B) Bronchodilators
C) Nitroglycerin
D) Inhaled steroids

Correct Answer: C (Nitroglycerin)

Explanation: Nitroglycerin is indicated for cardiac issues such as angina or myocardial infarction; it is not a treatment for asthma. Asthma is managed with bronchodilators and anti-inflammatories.

Q23. In topographic anatomy, the term "lateral" means:

A) Nearer the midline of the body.
B) Away from the midline of the body.
C) Nearer the head.
D) Away from the head.

Correct Answer: B (Away from the midline of the body.)

Explanation: In anatomical terms, 'medial' refers to structures closer to the midline, while 'lateral' refers to structures situated further away from the midline of the body.

Q24. Which one of the following conditions may mimic the signs of acute alcohol intoxication?

A) Hypoglycemia
B) Congestive heart failure.
C) Absence seizures.
D) Anaphylactic shock.

Correct Answer: A (Hypoglycemia)

Explanation: Hypoglycemia (low blood sugar) can cause symptoms like confusion, slurred speech, and ataxia, which are clinically similar to the signs of intoxication or a stroke.

Q25. Which one of the following breathing diseases is included in C.O.P.D.:

A) Croup.
B) Hyperventilation
C) Emphysema.
D) Dyspnea

Correct Answer: C (Emphysema.)

Explanation: Chronic Obstructive Pulmonary Disease (COPD) is an umbrella term that includes emphysema and chronic bronchitis. Croup and dyspnea are specific conditions or symptoms, not types of COPD.

Q26. A condition where air builds up in the pleural space, collapses the lung and puts pressure on the heart is called:

A) Closed pneumothorax
B) Tension pneumothorax
C) Hemothorax
D) Open pneumothorax

Correct Answer: B (Tension pneumothorax)

Explanation: A tension pneumothorax is a life-threatening emergency where air entering the pleural space cannot escape, increasing intrathoracic pressure and shifting the mediastinum, which compresses the heart and reduces cardiac output.

Q27. Oxygen humidification is recommended if you are administering oxygen for longer than:

A) 15 minutes.
B) 30 minutes.
C) 60 minutes.
D) 90 minutes.

Correct Answer: B (30 minutes.)

Explanation: Oxygen therapy is drying to the mucous membranes. Humidification is standard procedure when oxygen is administered for an extended duration, generally defined as longer than 30 minutes.

Q28. You find a male patient with obvious difficulty breathing. He is using his neck muscles, and is cyanotic. There are red blotches on his chest, and his neck is swelling. You suspect :

A) Anaphylaxis.
B) Bronchitis.
C) Emphysema.
D) Asthma.

Correct Answer: A (Anaphylaxis.)

Explanation: The combination of respiratory distress, cyanosis, and swelling of the neck/face is characteristic of a severe allergic reaction (anaphylaxis) due to upper airway obstruction and edema.

Q29. When you are giving mouth-to-nose AR, you should:

A) Hold the casualty's mouth closed.
B) Pinch the nostrils closed before blowing air into the casualty.
C) Tilt the head back less than for the mouth-to-mouth method of AR.
D) Keep the mouth and nose closed betweenbreaths.

Correct Answer: A (Hold the casualty's mouth closed.)

Explanation: When performing mouth-to-nose resuscitation, it is essential to hold the casualty's mouth closed to ensure that the air forced through the nostrils enters the lungs rather than escaping through the mouth.

Q30. During one-rescuer CPR for adults, the ratio of compressions to ventilations should be:

A) 5:1
B) 15:2
C) 30:2
D) 35:2

Correct Answer: C (30:2)

Explanation: Current international standards for adult CPR, as established by the ILCOR and AHA, specify a *compression-to-ventilation ratio of 30:2 for a single rescuer to ensure adequate circulation.*

Practice Set 3

Q1. A guideline for normal systolic blood pressure in an adult male would be:

A) 50 plus the man's age up to 150 mm. Hg.
B) 65 plus the man's age up to 120 mm. Hg.
C) 80 plus the man's age up to 130 mm. Hg.
D) 100 plus the man's age up to 150 mm. Hg.

Correct Answer: D (100 plus the man's age up to 150 mm. Hg.)

Explanation: A commonly used clinical guideline for estimating systolic pressure in adults is 100 plus the person's age, typically capped at approximately 150 mmHg.

Q2. A rapid body survey should take no longer than:

A) 30 seconds
B) 45 seconds
C) 60 seconds
D) 90 seconds

Correct Answer: A (30 seconds)

Explanation: The primary survey and rapid body survey must be concise to identify immediate life-threats. A duration of 30 seconds is the target to prioritize stabilizing the airway, breathing, and circulation.

Q3. To correctly size an oropharyngeal airway place the flange at the corner of the mouth with the tip reaching:

A) The angle of the patient's jaw
B) The top of the patient's ear
C) The patient's earlobe
D) Two fingerswidth from the flange.

Correct Answer: C (The patient's earlobe)

Explanation: To ensure an oropharyngeal airway is the correct size, the flange should rest against the lips, and the tip should reach the level of the earlobe or the angle of the jaw.

Q4. ASA should be offered to the patient with chest pain who is taking nitroglycerin:

A) Before the first dose of nitro
B) After the first dose of nitro
C) After the second dose of nitro
D) After the third dose of nitro

Correct Answer: B (After the first dose of nitro)

Explanation: Acetylsalicylic acid (ASA/aspirin) is commonly administered to cardiac patients during an active heart attack. It should be given after the initial assessment and the first dose of nitroglycerin to help prevent further clot formation.

Q5. When suctioning a non-breathing adult casualty, a first aider should:

A) Apply suction as soon as the suction tip touches the mouth.
B) Insert the suction tip deep into the larynx.
C) Suction for no more than 15 seconds.
D) Insert the tip with the curved side towards the tongue.

Correct Answer: C (Suction for no more than 15 seconds.)

Explanation: Suctioning is an invasive procedure that removes oxygen from the airway. Therefore, it is strictly limited to 15 seconds to prevent hypoxia and potential cardiac arrhythmias.

Q6. Which of the following devices provides the highest percentage of O2 delivery?

A) Nasal cannula.
B) Plastic face mask.
C) Partial rebreathing mask.
D) Non-rebreathing mask.

Correct Answer: D (Non-rebreathing mask.)

Explanation: A non-rebreathing mask with a reservoir bag allows for the delivery of high-concentration oxygen, typically providing between 80% to 90% oxygen, which is the highest among standard delivery masks.

Q7. You suspect a patient's pain is due to a myocardial infarction rather than angina because the pain:

A) Is under the sternum.
B) Radiates to the neck, jaw and arms.
C) Is not relieved by rest or medication.
D) Lasts for about 3 minutes.

Correct Answer: C (Is not relieved by rest or medication.)

Explanation: Angina is typically relieved by rest or nitroglycerin because it is caused by temporary myocardial ischemia. Pain that persists despite rest or medication suggests an ongoing myocardial infarction (heart attack).

Q8. A flail chest results when :

A) Three ribs are broken on each side of the chest.
B) Part of the spine becomes separated from the ribs.
C) The breastbone is broken in three places.
D) Several ribs are broken in more than one place.

Correct Answer: D (Several ribs are broken in more than one place.)

Explanation: A flail chest occurs when a segment of the rib cage breaks due to trauma and detaches from the rest of the chest wall. This happens when multiple ribs are fractured in at least two places.

Q9. Status epilepticus refers to:

A) Normal seizure pattern in epilepsy.
B) Seizures caused by a high fever.
C) Continuous seizure activity.
D) Seizures lasting longer than 2 minutes.

Correct Answer: C (Continuous seizure activity.)

Explanation: Status epilepticus is defined as a seizure lasting more than 5 minutes or a series of seizures where the patient does not regain consciousness between attacks; it is a medical emergency.

Q10. OPQRST is used to assess:

A) Pain
B) Level of consciousness
C) Vision
D) Hearing

Correct Answer: A (Pain)

Explanation: OPQRST is a standardized mnemonic used in emergency medicine to conduct a comprehensive assessment of a patient's pain. It stands for Onset, Provocation, Quality, Radiation, Severity, and Time.

Q11. The recommended oxygen flow rate for nasal cannulae (nasal prongs) is:

A) 1-6 litres/min.
B) 6-8 litres/min.
C) 8-12 litres/min.
D) 10-15 litres/min.

Correct Answer: A (1-6 litres/min.)

Explanation: A nasal cannula is a low-flow oxygen delivery system. The standard clinical recommendation for a nasal cannula is 1 to 6 litres per minute to effectively increase the fractional inspired oxygen concentration without causing mucosal irritation.

Q12. When aligning the head in the eyes-front neutral position, watch for:

A) Pulse, movement, sensation.
B) Crepitus, pain, resistance.
C) Pain, circulation, retention.
D) Tingling, numbness, paralysis.

Correct Answer: B (Crepitus, pain, resistance.)

Explanation: When performing spinal immobilization or aligning the head into a neutral position, the rescuer must monitor for signs of neurological impairment, specifically crepitus, increased pain, or physical resistance, which indicate potential spinal column compromise.

Q13. Manual support of the head and neck of the patient with suspected spinal injury can be released:

A) Once the breathing has been checked.
B) When a hard collar has been applied.
C) If the patient is unconscious.
D) When the patient is fully immobilized to a backboard.

Correct Answer: D (When the patient is fully immobilized to a backboard.)

Explanation: Manual stabilization of the head and neck is vital to prevent movement of an unstable spinal injury. This support must only be released once the patient is fully secured and immobilized to a rigid backboard or stretcher.

Q14. To prepare a completely amputated part of the body for transportation to hospital with the casualty, you should:

A) Place it in a clean plastic bag filled with cold water.
B) Wrap it in clean, moist dressings and keep it cool.
C) Wash it off and place it into a bag of crushed ice.
D) Wrap it in clean, moist dressings and keep it at body temperature.

Correct Answer: B (Wrap it in clean, moist dressings and keep it cool.)

Explanation: To preserve an amputated part, it should be wrapped in sterile, moist gauze or dressings, placed in a sealed plastic bag, and then placed in a container with cold water or a cooling pack. It should never be placed directly on ice, as this causes frostbite

Q15. Hypovolemic shock results from:

A) The body's reaction to a foreign protein.
B) An overdose of insulin.
C) Toxins produced by a severe infection.
D) Decreased volume of the blood.

Correct Answer: D (Decreased volume of the blood.)

Explanation: Hypovolemic shock occurs when there is a significant reduction in the volume of circulating blood or fluids, leading to inadequate tissue perfusion. This is commonly caused by severe hemorrhage, dehydration, or extensive burns.

Q16. A conscious casualty who has suffered a stroke becomes unconscious. To keep his airway open and help him breathe, you should:

A) Turn him into the recovery position with his unaffected side down.
B) Place him in a semisitting position.
C) Turn him into the recovery position with his unaffected side up.
D) Place him on his back.

Correct Answer: A (Turn him into the recovery position with his unaffected side down.)

Explanation: For an unconscious stroke patient, the recovery position is essential to keep the airway clear. Placing the patient on their unaffected side ensures that any secretions or vomit can drain away from the airway without obstruction.

Q17. A worker has fallen from a six-foot ladder onto a concrete floor. You first action should be.

A) Take a SAMPLE history.
B) Perform a secondary survey.
C) Take charge and perform a scene survey.
D) Asess the rate and quality of breathing.

Correct Answer: C (Take charge and perform a scene survey.)

Explanation: Before approaching any casualty, you must first perform a scene survey to ensure your own safety and the safety of others. Following the principles of ISPS and general safety, you must assess for hazards before beginning medical assistance.

Q18. To determine whether a c-spine injury has occurred, you should think about the Mechanism of Injury. This can best be described as:

A) The force that causes the injury and the way it acts on the body.
B) The circumstances after the injury.
C) The weather conditions at the time of the incident.
D) The area of the body that is injured

Correct Answer: A (The force that causes the injury and the way it acts on the body.)

Explanation: Mechanism of Injury (MOI) refers to the physical forces acting on the body during an event, such as a fall or impact. Analyzing the MOI helps determine the likely severity and nature of potential hidden injuries, including spinal trauma.

Q19. The correct ratio of compressions to ventilations for two -rescuer CPR on a child is:

A) 5:1
B) 15:2
C) 5:2
D) 15:1

Correct Answer: B (15:2)

Explanation: According to current guidelines for pediatric life support, for a child, the ratio of compressions to ventilations for two-rescuer CPR is 15:2. This provides a balance between circulating oxygenated blood and ensuring adequate ventilation.

Q20. Shock is best defined as:

A) Inadequate circulation to the body's tissues
B) Inadequate delivery of oxygen to the lungs
C) Inadequate elimination of waste products from the body.
D) Inadequate amount of insulin.

Correct Answer: A (Inadequate circulation to the body's tissues)

Explanation: Shock (hypoperfusion) is defined as a state where there is inadequate circulation of blood to the body's tissues. This prevents vital organs from receiving sufficient oxygen and nutrients to function correctly, leading to cellular failure.

Q21. The most critical factor in defibrillation is:

A) The time from collapse to defibrillation.
B) The skill of the AED responder.
C) The patient's previous cardiac history.
D) The type of defibrillator used.

Correct Answer: A (The time from collapse to defibrillation.)

Explanation: The most critical factor in successful defibrillation is the 'time to shock.' Early defibrillation significantly increases the chances of survival for a victim in ventricular fibrillation, as every minute delay reduces the success rate.

Q22. The AED will shock a patient:

A) With a sinus rhythm.
B) In asystole.
C) In ventricular fibrillation.
D) With pulseless electrical activity (PEA).

Correct Answer: C (In ventricular fibrillation.)

Explanation: An Automated External Defibrillator (AED) is designed to treat lethal, shockable heart rhythms. Ventricular fibrillation is the primary shockable rhythm that the AED is programmed to identify and treat effectively.

Q23. In the hypothermic patient you should limit the number of shocks to a maximum of:

A) 1.
B) 2.
C) 3.
D) 4.

Correct Answer: A (1.)

Explanation: In a severely hypothermic patient, the heart is extremely irritable. Current guidelines often limit defibrillation attempts to a single shock, followed by continuing CPR, to avoid exacerbating cardiac instability during the rewarming process.

Q24. When transporting a patient, a shock should be administered:

A) Only in a vehicle that is properly grounded.
B) Whenever the machine indicates shock necessary.
C) When the vehicle is stopped.
D) When the vehicle is travelling at less than 20 km per hour.

Correct Answer: C (When the vehicle is stopped.)

Explanation: Movement during defibrillation can cause artifacts and trigger false readings or impede the device's ability to analyze the rhythm correctly. For safety and accuracy, the vehicle must be brought to a complete stop before a shock is delivered.

Q25. When using an AED on a patient who is wearing a pacemaker, place the electrode pads:

A) Directly on pacemaker.
B) Two inches away from pacemaker.
C) Within one inch of the pacemaker.
D) At least one inch away from pacemaker.

Correct Answer: D (At least one inch away from pacemaker.)

Explanation: Electrode pads must be placed at least one inch away from any known implantable devices like pacemakers or ICDs. Placing pads directly over these devices can block the energy delivery or damage the internal electronics of the implant.

Q26. You have shocked a casualty and there are signs of circulation. You should:

A) Remove the AED from the scene.
B) Leave the AED attached to the casualty.
C) Disconnect the cables from the AED.
D) Remove the pads from the chest. 15 Burns

Correct Answer: B (Leave the AED attached to the casualty.)

Explanation: Once an AED is attached and signs of life return, you should leave the pads in place and keep the AED connected. The device may need to continue monitoring for cardiac rhythm stability, and the casualty may require further shocks if they re-arrest.

Q27. Burns that involve all layers of the skin are:

A) Superficial.
B) Partial thickness.
C) Full thickness.
D) First degree.

Correct Answer: C (Full thickness.)

Explanation: Full-thickness burns, historically known as third-degree burns, involve the destruction of all layers of the skin, including the epidermis and the dermis, often affecting underlying tissues as well.

Q28. In determining the amount of body surface burned, the area of the palm of the hand represents ____ % of the body surface.

A) One
B) Two
C) Five
D) Nine

Correct Answer: A (One)

Explanation: The Rule of Nines is often used for larger areas, but for smaller or scattered burns, the 'Rule of Palm' is the standard; the surface area of the casualty's palm represents approximately 1% of their total body surface area.

Q29. The most immediate serious complication associated with burns is:

A) Shock
B) Infection
C) Scarring
D) Hypothermia

Correct Answer: A (Shock)

Explanation: Hypovolemic shock is the most immediate life-threatening complication of severe burns due to the rapid loss of plasma and fluids through damaged skin, leading to systemic circulatory collapse.

Q30. A man is filling a gas tank on a generator when it bursts into flames. The caualty has a hoarse voice, he is blistered around the mouth and nose and both arms are reddened. You should classify this burn as:

A) Critical
B) Superficial
C) Moderate
D) Minor 16 Wound Management

Correct Answer: A (Critical)

Explanation: Burns involving the face (especially with inhalation signs like a hoarse voice or singed hairs) and those involving circumferential or deep tissue damage are classified as critical, requiring immediate medical evacuation and specialized hospital care.

Practice Set 4

Q1. An abrasion is:

A) deep break in the skin involving significant bleeding.
B) Partial or complete loss of a body part.
C) The result of a sharp object driven through soft tissue.
D) A scrape or rubbing away of the epidermis.

Correct Answer: D (A scrape or rubbing away of the epidermis.)

Explanation: An abrasion is a superficial wound caused by friction or scraping of the epidermis. While they may be painful due to exposed nerve endings, they usually involve minimal bleeding.

Q2. A soft tissue injury resulting from the impact of a blunt object is called:

A) laceration.
B) An avulsion.
C) A contusion.
D) A concussion.

Correct Answer: C (A contusion.)

Explanation: A contusion, commonly known as a bruise, occurs when blood vessels beneath the skin are damaged by the impact of a blunt object, causing internal bleeding without breaking the skin surface.

Q3. The acronym SHARP refers to:

A) The assessment of the severity of a wound injury.
B) Signs and symptoms used to determine the need to transport.
C) The depth of a penetrating wound and potential damage.
D) Signs of infection as assessed in wound injuries.

Correct Answer: D (Signs of infection as assessed in wound injuries.)

Explanation: In the context of wound management, the acronym SHARP-Swelling, Heat, Aches (Pain), Redness, and Pus-is used to identify the clinical signs of an infected wound.

Q4. Responders involved in wound management should consider:

A) Wounds should be thoroughly flushed to prevent infection.
B) All open wounds are contaminated to some degree.
C) The risk of infection continues until the wound is dressed and bandaged.
D) Gloves should be worn if you suspect the patient may be ill.

Correct Answer: B (All open wounds are contaminated to some degree.)

Explanation: Practicing universal precautions and wound management standards requires assuming that all open wounds are contaminated by bacteria from the environment, requiring proper cleaning and protective measures to prevent infection.

Q5. Tetanus is a condition:

A) That involves specific infection in the lower jaw.
B) That only occurs in the third world.
C) That is caused by a virus
D) That can be prevented by immunization.

Correct Answer: D (That can be prevented by immunization.)

Explanation: Tetanus is a serious bacterial infection that affects the nervous system and can lead to severe muscle contractions. It is entirely preventable through routine immunization and boosters.

Q6. Which one of the following factors increases the risk of life -threatening injury:

A) High velocity.
B) Low velocity.
C) High density.
D) Low energy.

Correct Answer: A (High velocity.)

Explanation: The energy transfer in an injury is proportional to the velocity squared ($KE = 1/2mv^2$). Therefore, high-velocity impacts significantly increase the risk of severe internal organ damage and life-threatening trauma.

Q7. Which action is part of first aid for a nosebleed?

A) Place an ice-pack on the back of the neck.
B) Plug the nose with gauze.
C) Lean forward and firmly pinch the soft parts of the nose.
D) Lean the casualty backwards in a sitting postion.

Correct Answer: C (Lean forward and firmly pinch the soft parts of the nose.)

Explanation: The correct first aid for a nosebleed is to have the casualty sit down and lean forward while pinching the soft part of the nose. Leaning backward is incorrect as it may lead to blood being swallowed or aspirated into the lungs.

Q8. To prevent further contamination and infection of a wound, you should cleanse the surrounding skin by:

A) Swabbing in circular motions around the wound.
B) Wiping lightly over the edges of the wound.
C) Swabbing from one side of the wound to the other.
D) Wiping away from the edges of the wound. 17 Heat and Cold Illness and Injury

Correct Answer: D (Wiping away from the edges of the wound. 17 Heat and Cold Illness and Injury)

Explanation: To prevent introducing further contamination, you should always cleanse the skin by wiping away from the edges of the wound. This prevents pushing debris from the surrounding skin into the injury.

Q9. The body loses heat in a variety of ways including:

A) radiation, conduction, evaporation and respiration.
B) radiation, refrigeration, evaporation and conduction.
C) conduction, convection, evaporation and refrigeration.
D) submersion, convection, radiation and respiration.

Correct Answer: A (radiation, conduction, evaporation and respiration.)

Explanation: The human body maintains homeostasis by losing heat through radiation, conduction, convection, evaporation, and respiration. These are the standard biophysical mechanisms of heat transfer.

Q10. If it becomes necessary to thaw a hand with deep frostbite you should ensure that:

A) The patient sees a doctor no later than 48 hrs after thawing
B) There is no danger of refreezing
C) Adequate pain relief medication is available
D) Capillary refill takes no longer than 15 seconds

Correct Answer: B (There is no danger of refreezing)

Explanation: If thawing frozen tissue, it is critical to ensure there is no chance of the area refreezing. Refreezing causes massive cellular destruction and irreversible damage to the tissue.

Q11. You have been called for an 81 year old unconscious male patient. He is in his living room. Despite the hot day, he is overly dressed. He is flushed and dry, extremely hot to touch. You suspect:

A) Heat exhaustion.
B) Stroke.
C) Hyperglycemia.
D) Heatstroke. Lifting and Carrying

Correct Answer: D (Heatstroke. Lifting and Carrying)

Explanation: Heatstroke is a medical emergency characterized by the body's inability to regulate temperature. A patient with hot, dry, flushed skin and altered mental status in a hot environment is exhibiting classic signs of heatstroke.

Q12. Body mechanics refers to:

A) Positioning and movement of the body
B) Number of rescuers needed to move a patient
C) A type of rescue carry
D) A special device to lift heavy loads

Correct Answer: A (Positioning and movement of the body)

Explanation: Body mechanics refers to the coordinated use of muscles, bones, and the nervous system to move, lift, and carry objects or people efficiently and safely while preventing strain on the caregiver.

Q13. You are alone and must remove a casualty with a suspected spinal injury from a very hazardous situation. You should

A) Grab his wrists and drag him lengthwise.
B) Keep his body rigid, support his head and neck and roll him away from the scene.
C) Tie his legs together and drag him feet first.
D) Grasp his clothing under his shoulders, support his head and neck, and drag him lengthwise.

Correct Answer: D (Grasp his clothing under his shoulders, support his head and neck, and drag him lengthwise.)

Explanation: In a hazardous scene where a spinal injury is suspected, the rescuer must prioritize moving the patient to safety. Dragging the patient lengthwise in the long axis of the body helps minimize spinal rotation and secondary injury.

Q14. To safely perform a chair carry you need

A) 2 rescuers
B) 3 rescuers
C) 4 rescuers
D) 5 rescuers 19 Musculoskeletal - Upper Limbs

Correct Answer: B (3 rescuers)

Explanation: A chair carry is a specialized technique that is most effective and stable when performed by three rescuers. One handles the head and neck, while the other two stabilize the chair and the patient's lower body.

Q15. Crepitus is:

A) . Poor distal circulation
B) A grating sound caused by bone ends rubbing together
C) A type of splint
D) A type of fracture

Correct Answer: B (A grating sound caused by bone ends rubbing together)

Explanation: Crepitus is the audible or palpable grating sensation caused by the rough ends of broken bones rubbing against each other during movement.

Q16. After discovering a possible fracture you should assess which of the following before and after splinting:

A) Sensation, morbidity, protrusion.
B) Capillary refill, pulse, sensation.
C) Pulse, motor function, sensation.
D) Crepitus, resistance, pain.

Correct Answer: C (Pulse, motor function, sensation.)

Explanation: Assessing neurovascular status-specifically Pulse, Motor function, and Sensation (PMS)-is mandatory before and after splinting to ensure that the splint has not compromised blood flow or nerve integrity.

Q17. A worker has had his hand caught in a car door. You suspect several fractured bones in the hand. The hand should be splinted:

A) Flat against the splint.
B) In a tight fist position.
C) With the fingers taped together.
D) In the position of function.

Correct Answer: D (In the position of function.)

Explanation: When splinting a hand, it should always be positioned in the 'position of function,' which is a natural, slightly curved *shape similar to holding a ball, to prevent stiffness and loss of dexterity.*

Q18. In the case of suspected fracture of the clavicle, emergency responders should use:

A) An arm sling supported with broad bandages.
B) A St. John tubular sling tied on the injured side.
C) A St. John tubular sling tied on the uninjured side.
D) Rigid splints to support the arm and support the shoulder.

Correct Answer: C (A St. John tubular sling tied on the uninjured side.)

Explanation: For a suspected clavicle fracture, a St. John tubular sling or similar support is used to keep the arm close to the body, with the sling tied on the uninjured side to distribute weight across the back.

Q19. Force on a joint may cause bone ends to come out of their proper position. This type of injury is called a:

A) Sprain
B) Fracture
C) Dislocation
D) Strain

Correct Answer: C (Dislocation)

Explanation: A dislocation occurs when the bone ends forming a joint are forced out of their normal anatomical alignment. This usually requires professional medical reduction and immobilization.

Q20. After an extremity fracture has been immobilized, the responder should check for circulation:

A) In the injured limb only.
B) Distal to the injury.
C) Proximal to the injury.
D) At the site of the fracture. 20 Musculoskeletal - Lower Limbs

Correct Answer: B (Distal to the injury.)

Explanation: After immobilization, it is vital to check the neurovascular status distal to the injury site. This ensures that the splinting process has not impaired circulation or nerve function in the hand or foot.

Q21. The longest, strongest bone in the body is the:

A) Humerus.
B) Fibula.
C) Tibia.
D) Femur.

Correct Answer: D (Femur.)

Explanation: The femur is the largest, longest, and strongest bone in the human body, extending from the hip to the knee.

Q22. Effective immobilization of the tibia includes immobilization of the:

A) Knee, tibia, fibula and ankle.
B) Femur, knee, tibia and ankle.
C) Femur and ankle.
D) Hip, femur, knee, tibia and ankle.

Correct Answer: A (Knee, tibia, fibula and ankle.)

Explanation: To effectively immobilize a tibia fracture, the joints immediately above and below the fracture must be secured. This includes the knee and the ankle joints.

Q23. You can best control the swelling and pain of an ankle sprain by:

A) Tight bandages and a heating pad.
B) Rest, immbolization, application of cold, and elevation.
C) Rigid splints and bandages
D) Compression, elevation and application of heat.

Correct Answer: B (Rest, immbolization, application of cold, and elevation.)

Explanation: The RICE protocol-Rest, Immobilization, Cold application, and Elevation-is the gold-standard first-aid treatment for sprains to reduce inflammation, swelling, and pain.

Q24. If the patient has an open fracture responders should :

A) Attempt to push bones back into the wound.
B) Use bulky dressings to pad around the protruding bones ends.
C) Apply pressure directly over the fracture to control bleeding.
D) Apply a tourniquet above the fracture site.

Correct Answer: B (Use bulky dressings to pad around the protruding bones ends.)

Explanation: With an open fracture, the primary goal is to prevent further infection. Bulky dressings should be applied around the protrusion; the bone must never be forced back, nor should pressure be applied directly over the bone end.

Q25. A traction splint could be used for which of the following injuries:

A) fractured pelvis.
B) An injured knee joint.
C) A dislocated hip.
D) A mid-shaft femur fracture.

Correct Answer: D (A mid-shaft femur fracture.)

Explanation: Traction splints are specifically designed to reduce pain and prevent further tissue damage in mid-shaft femur fractures by applying consistent tension to align the leg bones.

Q26. A concussion is best described as:

A) Bruising or swelling of the spinal cord.
B) Tearing of brain tissue.
C) Pooling of blood in the brain.
D) Temporary loss of brain function

Correct Answer: D (Temporary loss of brain function)

Explanation: A concussion is a mild traumatic brain injury resulting in a temporary disturbance of brain function, often without permanent structural damage, following a head impact.

Q27. You are examining the head of an infant who has been involved in a car crash. You need to be aware of:

A) Soft spots in the infant's skull.
B) Infant's pupils react differently than adults.
C) Whether or not the infant can cry forcefully.
D) The startle reflex.

Correct Answer: A (Soft spots in the infant's skull.)

Explanation: In infants, the presence of fontanelles (soft spots) means the skull is not fully ossified. Rescuers must be extremely careful during assessment and immobilization to avoid applying pressure to these areas.

Q28. When immobilizing a patient on a spine board, which part of the body is the first to be strapped?

A) Chest
B) Pelvis
C) Legs
D) Head

Correct Answer: A (Chest)

Explanation: When securing a patient to a spine board, the torso (chest/pelvis) should be strapped first to stabilize the body, followed by the head, to ensure the patient does not shift during the strapping process.

Q29. Contusions are:

A) Usually controlled with direct pressure
B) Very serious for the patient due to increased pressure in the brain.
C) Usually associated with scalp lacerations
D) Almost always seen in children.

Correct Answer: B (Very serious for the patient due to increased pressure in the brain.)

Explanation: Intracranial contusions are serious injuries where the brain surface is bruised, leading to swelling inside the confined space of the skull, which can cause rapidly increasing intracranial pressure.

Q30. The Glasgow Coma Scale measures three basic functions. They are:

A) Eye, verbal and motor responses.
B) Pulse rate, speech, involuntary movement.
C) Pulse rate, respiration rate, eye response.
D) Respiration rate, eye response, voluntary movement.

Correct Answer: A (Eye, verbal and motor responses.)

Explanation: The Glasgow Coma Scale (GCS) is a clinical tool used to objectively assess the level of consciousness. It evaluates three parameters: Eye opening response, Verbal response, and Motor response, providing a total score that indicates the severity of head injury.

Practice Set 5

Q1. Which one of the following changes in vital signs is characteristic of brain injury:

A) Increase in pulse rate.
B) Constant respiratory rate.
C) increase in blood pressure.
D) Decrease in blood pressure.

Correct Answer: C (increase in blood pressure.)

Explanation: Cushing's triad, a clinical manifestation of increased intracranial pressure due to brain injury, includes hypertension (increased blood pressure), bradycardia, and irregular respirations. While pulse often slows, an increase in blood pressure is a hallmark sign.

Q2. "Raccoon eyes" indicates:

A) Possible eye injury
B) Possible fracture of the jaw
C) Possible scalp laceration
D) Possible head injury

Correct Answer: D (Possible head injury)

Explanation: "Raccoon eyes" (periorbital ecchymosis) is a classic sign of a basilar skull fracture. The appearance of bruising around the eyes occurs as blood pools in the tissues following a fracture at the base of the cranium.

Q3. You are called to the scene of a motorcycle crash. The rider is dazed and walking around. You should:

A) Suspect spinal injury and manage accordingly.
B) Not worry about spinal injury because the person is mobile.
C) Advise the person to lie down in case of fainting.
D) Determine the chief complaint and provide first aid for it.

Correct Answer: A (Suspect spinal injury and manage accordingly.)

Explanation: In the event of a high-energy mechanism of injury like a motorcycle crash, spinal injury must always be assumed regardless of the patient's mobility. Immobilization protocols are critical until spinal injury can be clinically excluded to prevent neurological damage.

Q4. When managing a possible spinal injury:

A) The cervical immobilization device is applied by the police.
B) Transport the patient in the position of greatest comfort.
C) Apply a cervical immobilization device before assessing the patient.
D) Responders must provide initial stabilization by supporting the head and ensuring neutral alignment

Correct Answer: D (Responders must provide initial stabilization by supporting the head and ensuring neutral alignm

Explanation: The primary objective in managing a suspected spinal injury is to maintain the head in a neutral, in-line position. This stabilizes the cervical spine and prevents movement that could exacerbate potential cord injury during assessment and extrication.

Q5. A helmet must be removed from a patient:

A) If there are no airway or breathing problems.
B) If the patient will be immobilized to a long spinal immobilizaton device.
C) When the helmet has a face mask that interferes with the responder's ability to assist with ventilations
D) It is a full- face helmet.

Correct Answer: C (When the helmet has a face mask that interferes with the responder's ability to assist with ventila

Explanation: A helmet should only be removed if it prevents the rescuer from providing life-saving airway management or ventilation. Otherwise, the helmet provides effective stabilization of the head and should remain in place during immobilization.

Q6. When dealing with pelvic injuries you must always consider the possibility of:

A) Ruptured bladder.
B) Ruptured spleen.
C) Spinal injuries.
D) Rib fractures.

Correct Answer: C (Spinal injuries.)

Explanation: The pelvic ring protects internal structures; trauma causing pelvic fractures is often associated with significant force, which is frequently transmitted to the spine. Consequently, all pelvic injury patients must be managed with spinal precautions.

Q7. To properly measure a cervical collar, you:

A) Measure the distance from the ear lobe to the shoulder.
B) Measure the distance from the corner of the mouth to the ear lobe.
C) Measure the distance from the cheekbone to the shoulder blade.
D) Measure the distance from the trapezius muscle to the angle of the jaw 22 Poisons, Bites and Stings

Correct Answer: D (Measure the distance from the trapezius muscle to the angle of the jaw 22 Poisons, Bites and Stin

Explanation: A cervical collar must be sized correctly to ensure it provides adequate support without causing hyperextension or flexion. The standard measurement is the distance from the top of the trapezius muscle to the bottom of the chin (angle of the jaw).

Q8. Contact dermatitis occurs when:

A) The skin comes in contact with a poisonous substance.
B) A poison enters the eye of a contact lens wearer.
C) Hot gases are inhaled.
D) A poison is injected under the skin.

Correct Answer: A (The skin comes in contact with a poisonous substance.)

Explanation: Contact dermatitis is an inflammatory skin reaction caused by direct contact with an irritant or allergen. Onboard ships, this is often triggered by exposure to hazardous chemicals, paints, or specific marine flora/fauna.

Q9. Carbon monoxide:

A) May result from fires or automobile exhaust.
B) Has a very distinct odour
C) Is not life-threatening.
D) Requires the administration of low concentration O2.

Correct Answer: A (May result from fires or automobile exhaust.)

Explanation: Carbon monoxide is a toxic, colorless, and odorless gas commonly produced by incomplete combustion in engines or fires. Because it binds to hemoglobin more effectively than oxygen, it poses a severe, life-threatening risk of hypoxia.

Q10. A child has swallowed an unknown poisonous substance. You should:

A) Dilute the poison with several glasses of cool water.
B) Call your local Poison Control Centre and follow directions.
C) Give a solution of mild liquid dish detergent and water.
D) Use activated charcoal. 14 23 Anatomy and Physiology

Correct Answer: B (Call your local Poison Control Centre and follow directions.)

Explanation: In any suspected poisoning case, immediate contact with a professional Poison Control Centre is the standard procedure. They provide the most accurate medical guidance based on the specific substance and the casualty's age.

Q11. The external layer of the skin is called::

A) Cutaneous tissue.
B) Dermis.
C) Adipose tissue.
D) Epidermis.

Correct Answer: D (Epidermis.)

Explanation: The skin consists of three main layers: the epidermis (outermost), the dermis (middle/vascularized), and the subcutaneous/adipose tissue. The epidermis provides the primary barrier against environmental hazards and pathogens.

Q12. Muscles are attached to the bones by:

A) Ligaments.
B) Meninges.
C) Cartilage.
D) Tendons.

Correct Answer: D (Tendons.)

Explanation: Tendons are specialized fibrous connective tissues that anchor skeletal muscles to bones, allowing for force transmission and movement. In contrast, ligaments connect bone to bone to stabilize joints.

Q13. The appendix is located in which quadrant of the abdomen:

A) Lower right.
B) Upper right.
C) Upper left.
D) Lower left.

Correct Answer: A (Lower right.)

Explanation: The appendix is anatomically situated in the lower right quadrant (LRQ) of the abdomen. Pain or tenderness in this area is a significant indicator of potential appendicitis, a common surgical emergency.

Q14. The white exterior portion of the eye is called the:

A) Pupil.
B) Sclera.
C) Cornea.
D) Iris.

Correct Answer: B (Sclera.)

Explanation: The sclera is the dense, white, fibrous outer layer of the eye that provides structural integrity and protection. It surrounds the iris and pupil, protecting the internal contents of the globe.

Q15. A skeletal injury to the lower back could result from direct trauma to the:

A) Cervical vertebrae.
B) Thoracic vertebrae.
C) Lumbar vertebrae.
D) Coccygeal vertebrae.

Correct Answer: C (Lumbar vertebrae.)

Explanation: The lumbar vertebrae are located in the lower back region and support the majority of the body's weight. Direct trauma to this area frequently results in musculoskeletal injury or potential spinal cord complications.

Q16. The pulse point located in the upper portion of the thigh is called the:

A) Popliteal.
B) Femoral.
C) Brachial.
D) Temporal.

Correct Answer: B (Femoral.)

Explanation: The femoral artery is the primary pulse point located in the inguinal area (upper thigh). It is crucial for assessing circulation in the lower extremities and is a key site for controlling major hemorrhage.

Q17. Which one of the following is a function of the skin?

A) Regulation of oxygen levels.
B) Production of proteins.
C) Regulation of body temperature.
D) Removal of carbon dioxide.

Correct Answer: C (Regulation of body temperature.)

Explanation: The skin plays a vital role in thermoregulation by adjusting blood flow and promoting perspiration to dissipate heat. This is essential for maintaining systemic homeostasis in varying maritime working environments.

Q18. Urine is expelled from the body through the:

A) Kidneys.
B) Bladder.
C) Ureters.
D) Urethra.

Correct Answer: D (Urethra.)

Explanation: The urinary system filters blood through the kidneys, stores urine in the bladder, and expels it from the body via the urethra. The urethra serves as the final common pathway for the urinary system.

Q19. Insulin is produced in the:

A) Spleen.
B) Liver.
C) Pancreas.
D) Gall bladder.

Correct Answer: C (Pancreas.)

Explanation: Insulin is a hormone produced by the beta cells within the islets of Langerhans in the pancreas. It is essential for regulating glucose metabolism by facilitating the uptake of sugar into the cells.

Q20. When communicating with patients:

A) Stay at eye level & maintain eye contact.
B) Stay as close to the patient as possible.
C) Use medical terminology to enhance credibility.
D) Hide facts if the situation is serious.

Correct Answer: A (Stay at eye level & maintain eye contact.)

Explanation: Effective communication requires building rapport; staying at eye level shows respect and reduces anxiety. Maintaining eye contact signals attentiveness and ensures the patient feels heard during a medical emergency.

Q21. In cases of behavioural disorders, the responder should:

A) Always use restraints.
B) Give the patient a choice in the treatment
C) Play along with auditory or visual hallucinations.
D) Force the patient to make decisions.

Correct Answer: B (Give the patient a choice in the treatment)

Explanation: When treating behavioral disorders, maintaining the patient's dignity and autonomy is crucial. Giving the patient choices, whenever safe to do so, helps reduce escalation and encourages cooperation with the medical provider.

Q22. The best way to prevent a Critical Incident Stress situation from becoming worse is:

A) Ignore it.
B) Be familiar with the signs and symptoms.
C) Ensure early use of proper medication.
D) Force the responder to talk about the situation.

Correct Answer: B (Be familiar with the signs and symptoms.)

Explanation: Understanding the signs of Critical Incident Stress (CIS) is the best preventative measure, as it allows responders to recognize early warning signs in themselves or peers. This enables timely debriefing and support interventions.

Q23. Remove a contact lense only if:

A) There is a chemical burn to the eye.
B) The eyeball is injured.
C) Transport time is short.
D) The casualty is wearing hard lenses.

Correct Answer: A (There is a chemical burn to the eye.)

Explanation: Contact lenses should generally remain in place unless there is a chemical burn, where they may trap the chemical against the cornea. In such cases, removing the lens is critical to allow effective irrigation of the eye.

Q24. You discover a particle embedded in the casualty's cornea. You should:

A) Remove the particle with a moist corner of a facial tissue or clean cloth.
B) Flush the eye with water for 10 minutes.
C) Use a splinter forcep to remove the particle.
D) Cover the eye and transport the casualty to medical help.

Correct Answer: D (Cover the eye and transport the casualty to medical help.)

Explanation: Embedded particles in the cornea are dangerous to manipulate. Attempting to remove them can cause permanent vision damage; therefore, the eye should be covered to prevent further irritation and the patient transported for specialized care.

Q25. Signs and symptoms of intense light burns to the eyes include:

A) Bleeding from the conjunctiva.
B) Gritty feeling in the eyes.
C) Lacerations to the globe.
D) Discolouration of the orbits.

Correct Answer: B (Gritty feeling in the eyes.)

Explanation: Intense light burns (e.g., arc welding or severe UV exposure) cause inflammation of the corneal surface. A common symptomatic complaint from the patient is a persistent 'gritty' sensation, as if sand is trapped under the eyelids.

Q26. Eye injuries that include lacerations:

A) Should be covered with clean, moist dressings.
B) Iindicate possible skull fracture.
C) Usually involve a great deal of bleeding.
D) Require moderate pressure on the eye to control bleeding.

Correct Answer: C (Usually involve a great deal of bleeding.)

Explanation: Lacerations to the globe involve vascular tissues of the eye, which can lead to significant hemorrhage. Immediate, gentle, and non-pressure dressing is required to protect the integrity of the eye until medical consultation.

Q27. Irrigate eyes with:

A) Diluted vinegar.
B) Saline solution.
C) An appropriate chemical antidote.
D) Sodium bicarbonate.

Correct Answer: B (Saline solution.)

Explanation: Saline solution is the preferred medium for irrigating eyes as it is isotonic and least likely to cause further tissue irritation. It effectively flushes away debris or chemicals without disturbing the eye's natural pH balance.

Q28. Signs and symptoms of acute abdomen include:

A) Excessive hunger or thirst.
B) Hypertension and brachycardia.
C) Distension with rebound tenderness.
D) Fever with deep, shallow breathing.

Correct Answer: C (Distension with rebound tenderness.)

Explanation: The 'acute abdomen' is a surgical emergency characterized by severe, sudden onset abdominal pain. Rebound tenderness-pain upon release of pressure-is a hallmark clinical sign indicating potential peritonitis or inflammation.

Q29. Signs that a hernia is serious include:

A) Pain and tenderness at site.
B) Pale, ashen skin colour at site.
C) Hernia can be pushed back into body.
D) Hernia is above the diaphragm.

Correct Answer: A (Pain and tenderness at site.)

Explanation: A hernia becomes serious when it is 'strangulated' or incarcerated, cutting off blood supply. Pain and tenderness at the site are primary indicators that the tissues are compromised and require urgent medical intervention.

Q30. Which one of the following is not crucial in cases of abdominal distress:

A) Establish the cause of the pain.
B) Give oxygen by non-rebreather mask.
C) Be prepared to deal with vomiting.
D) Monitor the patient for signs of hypovolemic shock.

Correct Answer: A (Establish the cause of the pain.)

Explanation: While assessing the patient is important, the definitive diagnosis of abdominal pain is often impossible in the field. The priority is identifying life-threatening symptoms and shock, rather than diagnosing the specific anatomical cause of the distress.

Practice Set 6

Q1. In the case of abdominal evisceration, the emergency responder should:

A) Irrigate the wound before dressing it.
B) Give oxygen by nasal canula.
C) Dress the wound with a moist, sterile dressing
D) Gently reposition the organs in the abdominal cavity.

Correct Answer: C (Dress the wound with a moist, sterile dressing)

Explanation: For abdominal evisceration, organs must be protected from drying out and contamination. A sterile, non-adherent dressing moistened with saline provides an ideal environment while awaiting surgical intervention; never attempt to force organs back into the cavity.

Q2. "Referred pain":

A) Occurs when the sensation of pain is delayed.
B) Results when the two nervous systems come into contact.
C) Is the symptomatic description of pain by the patient.
D) Is the description of pain as recorded on the patient care report.

Correct Answer: B (Results when the two nervous systems come into contact.)

Explanation: Referred pain is a phenomenon where pain is perceived at a location different from the source of the injury. This occurs because the nerves from the organ and the skin share the same spinal cord pathways.

Q3. Trauma to male and female genitalia:

A) Often involves significant bleeding.
B) Is not usually painful because of protected nerve endings.
C) Usually results in sterility.
D) Often results in post-traumatic infection.

Correct Answer: A (Often involves significant bleeding.)

Explanation: Genital injuries are highly vascular and typically involve significant bleeding. While they are also extremely painful, the primary emergency concern remains the management of hemorrhage.

Q4. The priorities of MCIs include all but one of the following.

A) Overestimating the resources that may be required.
B) Comprehensive care of all patients.
C) Planning for the positioning of all vehicles.
D) Arranging for advanced level care providers at the scene.

Correct Answer: B (Comprehensive care of all patients.)

Explanation: In a Multiple Casualty Incident (MCI), resources are inherently limited. The goal is the 'greatest good for the greatest number,' meaning that comprehensive care of all patients individually is not possible due to triage constraints.

Q5. At the scene of a MCI:

A) All patients are assessed quickly and triaged.
B) Only conscious patients are assessed.
C) Patients without a detectable pulse are given the highest priority.
D) All patients are transported immediately.

Correct Answer: A (All patients are assessed quickly and triaged.)

Explanation: Triage is the process of rapidly sorting patients based on the severity of their injuries. This allows responders to identify those who need immediate intervention versus those whose care can be delayed without losing life.

Q6. Typically in a triage system:

A) Those whose survival requires immediate care are classified level 3.
B) Those with minor injuries but suffering extreme pain are classified level 1.
C) Those in cardiac arrest or not breathing are classified level 2.
D) Those who will survive if care is somewhat delayed are classified level 2.

Correct Answer: D (Those who will survive if care is somewhat delayed are classified level 2.)

Explanation: In standard triage systems, casualties are categorized by priority. Level 2 (Yellow) patients are those with serious injuries who can wait for a short duration while those with immediate life-threats (Level 1/Red) are managed first.

Q7. The most knowledgable responder in the first ambulance is generally assigned to role of:

A) Incident manager.
B) Triage officer.
C) Staging officer.
D) Communications officer.

Correct Answer: A (Incident manager.)

Explanation: The first qualified responder on the scene must assume the role of Incident Manager to establish command, initiate triage, and organize resources until more specialized support arrives.

Q8. Of the following casualties, which would you care for first? The casualty who is:

A) In shock without apparent injuries, but conscious.
B) Unconscious and lying on his back.
C) Bleeding from the forehead, but conscious.
D) Unconscious and lying on his stomach.

Correct Answer: B (Unconscious and lying on his back.)

Explanation: According to the ABC (Airway, Breathing, Circulation) priority, an unconscious patient on their back is at immediate risk of airway obstruction (tongue or secretions). Stabilizing the airway takes precedence over other non-life-threatening injuries.

Q9. The factor which indicates a drug related emergency is life-threatening is:

A) Altered mental status.
B) Dilated pupils slow to respond to light.
C) Lack of coordination and slurred speech.
D) High, low or irregular pulse.

Correct Answer: D (High, low or irregular pulse.)

Explanation: Irregular, abnormally high, or low pulses indicate cardiovascular or neurological instability, suggesting the drug may be causing systemic life-threatening failure rather than just behavioral changes.

Q10. Constant monitoring of the patient in a drug- related emergency is important because:

A) The patient will likely try to take more of the drug.
B) These patients are more likely to sue.
C) The patient's condition may change quickly.
D) The patient is more likely to become violent. 29 Communications

Correct Answer: C (The patient's condition may change quickly.)

Explanation: Patients in drug-related emergencies are unstable and susceptible to rapid deterioration in consciousness, respiratory rate, or hemodynamic status. Continuous monitoring is essential to catch these changes and initiate life support if necessary.

Q11. The most accurate source of time for reporting purposes is:

A) The responder's watch.
B) The dispatch unit.
C) The local radio station.
D) The patient's watch.

Correct Answer: B (The dispatch unit.)

Explanation: The dispatch unit maintains the official timeline of an emergency call. Using dispatch time ensures synchronization of *records between the shipboard team and shore-side medical services, which is critical for legal and medical documentation.*

Q12. An essential element of a responder's verbal report includes:

A) The name of the patient.
B) The patient's telephone number.
C) The time of the incident.
D) The patient's response to the care given

Correct Answer: D (The patient's response to the care given)

Explanation: A verbal report (e.g., MIST or ATMIST) must emphasize the patient's reaction to interventions performed. This informs the next level of care about the success of prior treatment and the patient's stability.

Q13. Which one of the following is the best way to assess dehydration in the elderly:

A) Check the mucous membranes in the eyes and mouth.
B) Check skin condition.
C) Take an oral temperature.
D) Assess the pulse rate.

Correct Answer: A (Check the mucous membranes in the eyes and mouth.)

Explanation: Dehydration in the elderly is best assessed by examining mucous membranes (mouth and eyes), which become dry and sticky as hydration levels fall. Skin turgor is less reliable in the elderly due to loss of skin elasticity.

Q14. An eighty-five year old lady has fallen down a flight of stairs. You suspect spinal injury and decide to put the patient on a spineboard. You should be aware that:

A) It may be difficult to fit a cervical collar.
B) Spinal curvature may make it difficult and uncomfortable to put the patient on the board.
C) A K.E.D. is better for this patient.
D) Because of her age, she may be more prone to vomitting.

Correct Answer: B (Spinal curvature may make it difficult and uncomfortable to put the patient on the board.)

Explanation: Elderly patients frequently suffer from kyphosis (spinal curvature). Placing them on a rigid spineboard can be dangerous if the voids created by the curvature are not adequately padded, causing discomfort and potential secondary spinal injury.

Q15. Which of the following is NOT a special consideration when assessing elderly patients:

A) Geriatric patients often have a reduced gag reflex.
B) Radial pulses may be weakened by peripheral vascular disease.
C) Patients may not show signs of fever even in cases of serious infection.
D) Mental status often reflects a fear of emergency responders. 31 Emergency Childbirth

Correct Answer: D (Mental status often reflects a fear of emergency responders. 31 Emergency Childbirth)

Explanation: Elderly patients may indeed have diminished physiological responses, such as a lack of fever, weakened pulses, or reduced gag reflexes. However, their mental status often reflects underlying medical conditions rather than a simple 'fear of responders'.

Q16. Crowning occurs:

A) In the first stage of labour.
B) In the second stage of labour.
C) In the third stage of labour.
D) In the fourth stage of pregnancy.

Correct Answer: B (In the second stage of labour.)

Explanation: Crowning is the stage where the baby's head becomes visible at the vaginal opening. This indicates that birth is imminent and the second stage of labor (expulsion) is occurring.

Q17. When suctioning a newborn you should use:

A) bulb syringe.
B) A V-Vac suction device.
C) A straw.
D) A battery powererd suction device.

Correct Answer: A (bulb syringe.)

Explanation: A bulb syringe is the safest and most effective tool for clearing mucus from the newborn's mouth and nose. It allows for controlled suction without damaging the delicate tissues of the infant's airway.

Q18. To help control bleeding after the baby is born:

A) Pack the vagina with pads.
B) Massage the mother's uterus.
C) Elevate the mother's legs above heart level.
D) Place the mother on her right side. 32 Pediatric Emergencies

Correct Answer: B (Massage the mother's uterus.)

Explanation: Post-partum hemorrhage is often caused by uterine atony. Massaging the uterus stimulates it to contract, which constricts the bleeding blood vessels at the placental site and is the primary intervention for the mother.

Q19. One of the most difficult things to assess in infants and young children is:

A) Blood pressure.
B) Pain.
C) Adequate perfusion.
D) Level of consciousness.

Correct Answer: B (Pain.)

Explanation: Assessing pain in infants and young children is notoriously difficult because they lack the verbal skills to describe it. Responders must rely on observation, such as crying, facial expressions, and body language, to infer the level of distress.

Q20. The onset of shock in infants and children:

A) May be sudden and severe.
B) Is no different than adults.
C) Usually progresses slowly.
D) Usually results from cardiac problems.

Correct Answer: A (May be sudden and severe.)

Explanation: Infants and children possess superior physiological compensatory mechanisms that can mask shock symptoms until the child's body suddenly decompensates. Consequently, when clinical signs of shock appear, the progression is often rapid, sudden, and severe, necessitating immediate intervention.

Q21. Assessments of children should take into consideration:

A) No splints should be used since all are designed for adult patients.
B) Abdominal injuries are less serious since muscles are less developed.
C) The skull has not completely fused, so head injuries are uncommon.
D) Chest injury is likely to involve organs without damage to ribs.

Correct Answer: D (Chest injury is likely to involve organs without damage to ribs.)

Explanation: Children have highly elastic chest walls and less developed musculature. This anatomy allows significant force to be transmitted to internal organs (like the heart or lungs) during impact, often resulting in severe internal organ injury even in the absence of visible rib fractures.

Q22. The responder who suspects a child to be the victim of abuse should:

A) Question parents immediately.
B) Focus on management of injuries to the child.
C) Gather evidence and question bystanders.
D) Immediately remove the child from parents or caregivers.

Correct Answer: B (Focus on management of injuries to the child.)

Explanation: When suspecting child abuse, the primary focus must remain on the medical stabilization and management of the child's injuries. Questioning parents or gathering evidence is a matter for law enforcement and child protective services, not the first-aid responder, to avoid compromising the patient's care or legal integrity.

Q23. A child with history of a sore throat, fever and painful swallowing, has breathing difficulties. You should:

A) Arrange for immediate transport to a medical facility.
B) Start mouth-to-mouth artificial respiration.
C) Stand by and encourage coughing.
D) Begin abdominal thrusts.

Correct Answer: A (Arrange for immediate transport to a medical facility.)

Explanation: A child presenting with a sore throat, fever, and difficulty breathing is highly suggestive of epiglottitis, a life-threatening airway obstruction. This is a medical emergency requiring rapid transport to a facility with advanced airway management capabilities; intervention attempts like back blows are contraindicated.

Q24. The mammalian reflex refers to:

A) An increase in heart rate and respiration.
B) A decrease in heart rate and dilation of blood vessels.
C) An increase in heart rate and constriction of pupils.
D) A decrease in heart rate and constriction of blood vessels.

Correct Answer: D (A decrease in heart rate and constriction of blood vessels.)

Explanation: The mammalian diving reflex is a physiological survival response triggered by cold water contact with the face. It causes bradycardia (decreased heart rate) and peripheral vasoconstriction (narrowing of blood vessels) to redirect oxygenated blood to essential organs like the brain and heart.

Q25. Three common scuba related emergencies are:

A) Air embolism, decompression sickness and mammalian reflex.
B) Air embolism, decompression sickness and barotrauma.
C) Decompression sickness, the bends and barotrauma.
D) Air embolism, barotrauma and the squeeze.

Correct Answer: B (Air embolism, decompression sickness and barotrauma.)

Explanation: Air embolism, decompression sickness (the bends), and barotrauma are the three primary emergencies associated with scuba diving. These conditions are caused by rapid pressure changes and gas expansion/contraction within the body's tissues and air-filled cavities.

Q26. In cases of near drowning:

A) The lungs are often filled with water and require deep suctioning.
B) Laryngospasm prevents water from entering the lungs.
C) Abdominal thrusts should always be performed before ventilating the patient.
D) Manual suction devices are not powerful enough to remove water and fluids from the airway.

Correct Answer: B (Laryngospasm prevents water from entering the lungs.)

Explanation: Laryngospasm is a protective reflex where the vocal cords spasm to prevent water entry into the trachea. In most near-drowning incidents, this spasm prevents significant amounts of water from actually entering the lungs, despite the patient appearing to have aspirated.

Q27. You have been helping at a water rescue when one of the divers begins to have problems. You suspect air embolism. This patient should receive:

A) Oxygen at 15 lpm.
B) Oxygen at 6 lpm.
C) Epinepherine.
D) Inhaled steroids.

Correct Answer: A (Oxygen at 15 lpm.)

Explanation: Arterial Gas Embolism (AGE) is a critical dive-related emergency caused by gas bubbles entering the bloodstream. High-flow oxygen (15 lpm) via a non-rebreather mask is the definitive initial field treatment to help reduce bubble size and improve tissue oxygenation.

Q28. To reduce the risk of respiratory infection and pneumonia where medical care is delayed, responders should:

A) Have the patient breathe deeply and cough.
B) Provide the patient with adequate hydration.
C) Position the patient in the position of comfort.
D) Ensure the patient is kept warm and at rest.

Correct Answer: A (Have the patient breathe deeply and cough.)

Explanation: Encouraging a patient to breathe deeply and cough regularly helps clear pulmonary secretions and prevents the collapse of lung tissue (atelectasis). This is a critical nursing intervention to minimize the risk of pneumonia in patients during prolonged transport or delayed medical care.

Q29. When positioning a patient, responders should consider that:

A) Raising arms and legs will produce swelling.
B) Semi-sitting will improve blood flow to the heart.
C) Knees raised will reduce pressure on the abdomen.
D) Recovery position will often improve breathing.

Correct Answer: C (Knees raised will reduce pressure on the abdomen.)

Explanation: Placing a patient with abdominal pain or injury in a semi-fowler's position with the knees flexed helps relax the abdominal musculature. This relaxation reduces tension and pressure on the abdominal wall, significantly increasing the patient's comfort.

Q30. When caring for wounds over extended periods, always:

A) Remove dressings and bandages regularly to check the wound.
B) Clean wounds with antiseptic before applying dressings.
C) Apply a tourniquet to prevent excessive bleeding.
D) Check regularly for signs of infection or reduced circulation.

Correct Answer: D (Check regularly for signs of infection or reduced circulation.)

Explanation: When managing wounds over extended durations, regular assessment for circulation, sensation, and movement (CSM) is vital. Checking for signs of secondary infection or constriction caused by bandages is essential for effective long-term wound healing and safety.

Practice Set 7

Q1. The maintenance of fluids and nutrition is key to extended patient care and can be provided by:

A) Giving fluids and food to all patients regardless of MOI.
B) Use small amounts of liquids and foods to determine whether the patient can tolerate the intake.
C) Allow the patient to consume any foods they would like to make them feel more comfortable.
D) Even if the patient is not thirsty or hungry the responder must insist on intake.

Correct Answer: B (Use small amounts of liquids and foods to determine whether the patient can tolerate the intake.)

Explanation: If medical evacuation is delayed, small, frequent amounts of clear liquids help determine tolerance. Large volumes of fluids or food should be avoided, especially if surgery or anesthesia might be required upon arrival at a medical facility, to minimize the risk of aspiration.

Q2. When administering medication, responders should consider that:

A) Right medication, right amount, right person, right time, right method.
B) Right nutrition, , right method, right time, right person, right amount
C) Right medication, right action, right person, right time, right responsibility
D) Right medication, right amount, right person, right responsibility, right action

Correct Answer: A (Right medication, right amount, right person, right time, right method.)

Explanation: The 'five rights' of medication administration are a global standard to ensure patient safety. They are the right medication, right amount, right person, right time, and right method of administration.

Q3. To give ear drops to an adult, the ear should be pulled:

A) Downward and backward.
B) Downward and forward.
C) Upward and backward.
D) Upward and forward.

Correct Answer: C (Upward and backward.)

Explanation: When administering ear drops to an adult, the ear canal should be straightened by pulling the pinna (auricle) upward and backward. This technique aligns the canal for better penetration of the medication toward the tympanic membrane.

Q4. Provincial first aid legislation prescribes the contents of:

A) First aid boxes, first aid rooms and first aid manuals.
B) First aid stations, training programs and equipment manufacturers.
C) First aid boxes, first aid reports and record keeping.
D) First aid boxes, first aid rooms and training.

Correct Answer: D (First aid boxes, first aid rooms and training.)

Explanation: Provincial or national workplace safety legislation typically mandates the provision and contents of first-aid kits, the standards for first-aid rooms, and the requirements for certified first-aid training. These regulations ensure a baseline standard of emergency care at work.

Q5. First aid stations must:

A) Be easily accessible to workers
B) Have a shower
C) Be at least 6 square meters in size
D) Have oxygen available Ambulance Operation/Maintenance

Correct Answer: A (Be easily accessible to workers)

Explanation: First aid stations must be located where they are easily accessible to workers at all times. Quick access is critical during a medical emergency to ensure that life-saving treatment can be initiated without unnecessary delays.

Q6. Footprints can best be described as:

A) The area of contact between the road surface and the tires.
B) The type of tread on the tire.
C) Footsteps used to get into the back of the ambulance.
D) Skid patterns on dry pavement.

Correct Answer: A (The area of contact between the road surface and the tires.)

Explanation: In the context of vehicle operations, 'footprint' refers to the patch of the tire that is in direct contact with the road surface at any given moment. This contact area is crucial for traction, steering, and braking efficiency.

Q7. Ground guides or spotters should be situated:

A) Behind the vehicle on the driver side.
B) On the front left side of the vehicle.
C) Front and centre of the vehicle.
D) On the front right side of the vehicle.

Correct Answer: A (Behind the vehicle on the driver side.)

Explanation: Ground guides or spotters should always be positioned where they can clearly see the vehicle driver and be seen in the driver's mirrors, typically behind the vehicle on the driver's side. This ensures constant visual communication during maneuvers.

Q8. When completing the patient narrative:

A) Try to establish an accurate diagnosis.
B) Record the signs and symptoms as accurately as possible.
C) Do not include negative data.
D) Always use medical terminology.

Correct Answer: B (Record the signs and symptoms as accurately as possible.)

Explanation: Patient care reports or narratives should be strictly objective, recording observed signs (objective) and reported symptoms (subjective) as accurately as possible. The goal is to provide a clear, factual account rather than attempting a diagnostic conclusion.

Q9. A patient refuses your care. You should:

A) Fill out and personally sign a "Refusal of Care" form.
B) Proceed with patient assessment and management and ignore the refusal.
C) Find a relative or family member to give you consent.
D) Have the patient and a witness sign the "Refusal of Care" form.

Correct Answer: D (Have the patient and a witness sign the "Refusal of Care" form.)

Explanation: When a competent adult refuses care, it is legally imperative to document the refusal properly to protect the responder. Having the patient and a witness sign a 'Refusal of Care' form validates that the risks of refusal were explained and understood.

Q10. Sublingual medication is given:

A) By injection using an auto-injector.
B) Under the patient's tongue.
C) By inhalation using a metered dose inhaler.
D) As a tablet to be swallowed.

Correct Answer: B (Under the patient's tongue.)

Explanation: Sublingual medication is designed to be placed under the tongue, where it is quickly absorbed into the bloodstream through the rich network of capillaries in the sublingual mucosa, bypassing the digestive system for a rapid effect.

Q11. Contraindications of a medication tell responders:

A) When a medication should not be given to the patient.
B) When the medication is commonly used for patients.
C) The expected results of the medication.
D) Actions that might not be desirable yet occur along with desired effects.

Correct Answer: A (When a medication should not be given to the patient.)

Explanation: Contraindications are conditions or factors that serve as reasons to withhold a specific medication. They indicate when a drug could be harmful to a patient rather than beneficial, based on medical history or current symptoms.

Q12. The ACTION of the drug refers to:

A) The harmful effects.
B) The side effects.
C) The expected effects.
D) The method of administration.

Correct Answer: C (The expected effects.)

Explanation: The action of a drug refers to the desired therapeutic effect it is expected to produce in the body. This is the physiological change that the medication is intended to achieve to treat the patient's condition.

Q13. To manage severe toothaches, responders should:

A) Advise the patient to chew on the other side of the mouth.
B) Advise the patient to drink hot liquids to sooth discomfort.
C) Advise the patient to suck on ice cubes to dull the pain.
D) Extract the tooth with the assistance of a second responder.

Correct Answer: A (Advise the patient to chew on the other side of the mouth.)

Explanation: To manage severe toothache, the patient should be advised to avoid chewing on the affected side to prevent further irritation or damage. This minimizes mechanical stress on the damaged tooth and associated inflamed nerves.

Q14. When handling a knocked-out tooth you should:

A) Place the tooth back in the patient's mouth.
B) Place the tooth in a cup of milk.
C) Wrap the tooth in dry gauze.
D) Place the tooth in a mild acidic solution using vinegar.

Correct Answer: B (Place the tooth in a cup of milk.)

Explanation: A knocked-out (avulsed) tooth is best preserved in milk, which provides a balanced pH and nutrient environment that helps keep the periodontal ligament cells alive. This significantly increases the success rate of reimplantation if performed promptly.

Q15. Lividity refers to:

A) Stiffening of the joints post mortem.
B) Cooling of the body post mortem.
C) Settling of blood in the body due to gravity.
D) The beginning signs of decomposition.

Correct Answer: C (Settling of blood in the body due to gravity.)

Explanation: Lividity (livor mortis) is a post-mortem change characterized by the settling of blood in the lower (dependent) parts of the body due to gravity. This occurs because the heart is no longer pumping to circulate blood.

Q16. Later signs of death include:

A) Staining, milky corneas and flushed skin.
B) Staining, lividity and decomposition.
C) Rigor mortis, lividity and milky corneas.
D) Rigor mortis, flushed skin and decomposition.

Correct Answer: C (Rigor mortis, lividity and milky corneas.)

Explanation: Rigor mortis (stiffening of muscles), livor mortis (lividity/blood settling), and cloudy or milky corneas (due to corneal dehydration) are definitive late signs of death. Flushed skin is not a sign of death.